

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	KEYMAKER-U01 Substudy 01E: A Phase 2 Umbrella Study With Rolling Arms of Investigational Agents With or Without Chemotherapy in Combination With Pembrolizumab in Treatment of Participants With Newly Diagnosed Resectable Stages II-IIIB (N2) Non-small Cell Lung Cancer (NSCLC)
Short Title:	Pembrolizumab (MK-3475) Plus Investigational Agents in Resectable Non-small Cell Lung Cancer (NSCLC) (MK-3475-01E/KEYMAKER-U01)
Protocol Number:	906228081365110469
NCT Number:	NCT06788912
Sponsor Name:	Merck
Investigational Product:	paracetamol
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Pathological Complete Response (pCR): pCR is defined as absence of residual viable invasive cancer on hematoxylin- and eosin-stained slides of the resected lung specimen and lymph nodes. 2. Percent Residual Viable Tumor (%RVT): %RVT is defined as the percentage of residual tumor estimated by comparing the estimated cross-sectional area of viable tumor with estimated cross-sectional areas of remainder of tumor bed. The tumor bed is defined as the area of tissue occupied by viable tumor or tumoral regression (includes areas of necrosis, foamy macrophages, giant cell reaction, cholesterol cleft granuloma, and inflammation.)
Secondary Obj.:	1. Percentage of Participants Who Report at Least 1 Adverse Event (AE) 2. Percentage of Participants Who Discontinue Study Treatment Due to an AE 3. Event-free Survival (EFS)

2.2 Background & Rationale

The main goals are after treatment given before surgery, to measure the number of people who have no signs of cancer cells in tumors and lymph nodes removed during surgery; and to learn about whether the cancer gets smaller or goes away by measuring the number of people with a certain number of living cancer cells in the tumor removed during surgery.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: RECRUITING
Target N:	60

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

- * Has previously untreated and pathologically confirmed resectable Stage II, IIIA, or IIIB (N2) non-small cell lung cancer (NSCLC)
- * Able to undergo protocol therapy, including necessary surgery
- * Confirmation that epidermal growth factor receptor (EGFR) -directed therapy is not indicated as primary therapy
- * Has an Eastern Cooperative Oncology Group (ECOG) performance status of either 0 or 1 as assessed within 10 days before initiation of study intervention.
- * Is able to provide archival or newly obtained core/excisional biopsy of the primary lung tumor or lymph node metastasis.

4.2 Exclusion Criteria

- * Has one of the following tumor locations/types: NSCLC involving the superior sulcus, large-cell neuro-endocrine cancer, mixed tumors containing small cell and non-small cell elements, or sarcomatoid tumor.
- * Has Grade ?2 peripheral neuropathy.
- * Has history of documented severe dry eye syndrome, severe Meibomian gland disease and/or blepharitis, or severe corneal disease that prevents/delays corneal healing.
- * Has active inflammatory bowel disease requiring immunosuppressive medication or previous history of inflammatory bowel disease (eg, Crohn's disease, ulcerative colitis, or chronic diarrhea).
- * Has uncontrolled, significant cardiovascular disease or cerebrovascular disease.
- * Received a live or live-attenuated vaccine within 30 days before the first dose of study intervention
- * Received prior radiotherapy within 2 weeks of start of study intervention, or radiation related toxicities, requiring corticosteroids.
- * Diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy (in dosing exceeding 10 mg daily of prednisone equivalent) or any other form of immunosuppressive therapy within 7 days prior to the first dose of study intervention.
- * Known additional malignancy that is progressing or has required active treatment within the past 5 years.
- * Severe hypersensitivity (?Grade 3) to pembrolizumab and/or any of its excipients.
- * Active autoimmune disease that has required systemic treatment in the past 2 years. Replacement therapy (e.g. thyroxine, insulin, or physiologic corticosteroid) is allowed.
- * History of (noninfectious) pneumonitis/interstitial lung disease that required steroids or has current pneumonitis/interstitial lung disease.
- * Active infection requiring systemic therapy.
- * Hepatitis B (defined as hepatitis B surface antigen \[HBsAg\] reactive) or Hepatitis C virus (defined as detectable hepatitis C virus (HCV) ribonucleic acid (RNA) \[qualitative\]) infection.
- * Known history of human immunodeficiency virus (HIV) infection.
- * History of allogeneic tissue/solid organ transplant.

11. Study Identifiers & Governance

Primary ID: 906228081365110469

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Registry Number: NCT06788912

Sponsor: Merck

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15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____